Government Policies: Health and Public Health

Official Health-Service Supplement (checked 14 October 2026)

These notes use official resources discovered through the National Portal's Health and Wellness category. Source identifiers resolve to URLs in docs/corpus_sources.json. They describe programme services, not individual medical advice. Existing local training codes, including NTR-7, are not established as government-prescribed forms by these sources.

PM-JAY: Hospital Cover and Access [GOV-PMJAY]

Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB PM-JAY) provides eligible families health cover up to Rs 5 lakh per family per year for secondary and tertiary care hospitalisation in empanelled public and private hospitals. Benefits are cashless and portable across the country within the empanelled network. The annual amount is a family cover limit, not a cash transfer or an amount payable to every family member. Applicable eligibility, treatment-package rules and hospital empanelment must be checked. Creating a digital health account is a different service from establishing PM-JAY entitlement; see the ABHA notes in the digital-services document and senior-citizen coverage in the social-security document.

eSanjeevani: Two Telemedicine Routes [GOV-ESANJEEVANI]

The Ministry of Health and Family Welfare describes two eSanjeevani models. Provider-to-provider telemedicine supports assisted consultations at Ayushman Arogya Mandirs, formerly Health and Wellness Centres, linking frontline providers with doctors and specialists at higher-level hubs. Patient-to-provider telemedicine, eSanjeevaniOPD, supports remote outpatient consultations from home using devices such as smartphones or laptops. For corpus reporting exercises, count registration, a consultation completed and any subsequent referral or follow-up separately; this reporting distinction is editorial guidance.

U-WIN: Immunisation Records [GOV-UWIN]

The Ministry's 29 November 2024 programme description identifies U-WIN as the digital platform for Universal Immunization Programme services for pregnant women and children from birth to 16 years. It supports vaccination records, appointment-related citizen services, SMS reminders, QR-based vaccination certificates and offline recording by vaccinators where connectivity is unavailable. Registration, an appointment booked, a recorded dose and completion of the age-appropriate schedule are distinct stages in this corpus's reporting exercises. These notes do not reproduce a vaccine schedule; use the current official National Immunization Schedule for dose-specific information.

Policy Objectives

Health policy aims to improve access to preventive, primary, secondary and tertiary healthcare. Public health programmes also address maternal and child health, infectious diseases, nutrition, medicines, health infrastructure and the healthcare workforce.

Ayushman Bharat

Ayushman Bharat is a major policy framework associated with expanding health coverage. Its components include strengthening primary healthcare through Ayushman Arogya Mandirs (formerly Health and Wellness Centres) and financial protection for eligible beneficiaries through the PM-JAY health assurance component.

National Health Mission

The National Health Mission supports public health systems and works with States and Union Territories on healthcare delivery. Areas of attention include maternal health, child health, disease control, human resources and strengthening facilities.

Digital Health

Digital health policy promotes interoperable systems and improved access to health services and records. Digital platforms can support telemedicine, appointments, health information and service delivery, while privacy and secure handling of personal information remain important considerations.

Prevention and Primary Care

Public policy increasingly emphasizes prevention, screening, immunisation and early diagnosis. Primary healthcare can reduce pressure on higher-level hospitals when common conditions are managed earlier and referral systems function effectively.

Implementation Challenges

Health is a large and diverse sector. Outcomes depend on financing, State capacity, availability of doctors and nurses, medicines, infrastructure, geographic access and public awareness. Policy design therefore needs strong implementation and monitoring.

Service and Outcome Measures

Health-system reporting may count facilities opened, facilities functional, patients registered, consultations completed or referrals completed. These are not interchangeable measures of improved health access. A telemedicine referral does not necessarily mean that a specialist consultation occurred, and a person covered by an assurance scheme may still face travel, documentation or hospital availability barriers. Monitoring also needs to distinguish reported disease incidence from improved detection, since expanded screening can temporarily increase the number of recorded cases.

Public-health teams may use water-quality alerts, sanitation conditions and local disease surveillance to prioritise outreach. They must not infer that a household connection, a sanitation certificate, or a nutrition registration by itself establishes safe water use, disease prevention or a completed health outcome.

When a family is referred between a nutrition centre and a health facility, the minimum information needed for continuity of care may be reconciled through approved service processes. Aggregate reporting must distinguish children screened, children referred,

children assessed and children who completed follow-up.

Government Policies: Nutrition, Early Childhood and Care

Policy Purpose

Nutrition and early-childhood policy supports maternal nutrition, child growth, responsive care, early learning and timely referral to health services. Delivery is designed around the first 1,000 days and continued support for young children, while recognising that a service contact alone does not prove an outcome.

Growth Monitoring and Referral

Community nutrition sessions may measure growth and screen children for nutrition risk. The reporting chain must distinguish children screened, children identified for follow-up, referrals issued, referrals accepted by a health facility, children clinically assessed and children completing follow-up. A screening result is not a clinical diagnosis, and a referral is not proof that treatment was received.

The Nutrition Referral Tracker (NTR-7) is used when a nutrition centre hands a child or mother to a health facility. Each NTR-7 record must include the referral reason, issue date, receiving service point, acceptance status, correction status and follow-up due date. NTR-7 measures care coordination; it is not a treatment-completion certificate.

Service Package and Inclusion

Frontline workers may provide counselling on infant and young-child feeding, maternal diets, hygiene and responsive caregiving, alongside age-appropriate supplementary nutrition where the programme provides it. Home visits and centre-based sessions should offer assisted access for families affected by disability, migration, language barriers, care burdens or limited connectivity. Families should have a clear local route to ask for a missed entitlement or correct a record.

Convergence with Health

Nutrition centres should use an approved referral pathway for children or mothers who require clinical assessment. Health facilities retain responsibility for recording diagnosis, treatment and follow-up. Joint reviews may reconcile the minimum information needed to identify pending referrals, but they must not collapse nutrition screening, health assessment and completed treatment into one beneficiary count.

Responsible Digital Records

Digital records should retain the service stage, date, local service point and reason for any pending correction. Assisted entry is valid delivery support, but a successful form submission is not proof that a family received nutrition, health care or cash support. Any reconciliation across services should be purpose-limited, auditable and supported by a correction process that preserves the original discrepancy.

Measurement and Accountability

District reporting should identify whether it counts children, visits, food-service days, screenings, referrals, assessments or completed follow-ups. An increase in registrations may reflect outreach or duplicate cleanup rather than improved nutritional status. Outcome claims should use the stated measurement method, reporting period and denominator.

Common Administration and Evidence Standards

Use a written procedure with a named responsible authority, intended users, required records, completion criteria and routes for exceptions or clarification. Coordinate policy authorities, field offices, local institutions and delivery partners, retaining visible pending dependencies. Address distance, documentation, language, disability, connectivity and cost through outreach and assisted access. Distinguish administrative activity from actual outcomes, and record the unit, date, source, status and unresolved cases.

Health Referral Status and Reporting

Health and nutrition referral reporting must keep referrals issued, referrals accepted, clinical assessments, treatment where relevant and completed follow-up separate. State the reporting period and denominator. NTR-7 tracks referral coordination; health records establish assessment and treatment. Reconcile only the minimum necessary information, retain the original discrepancy and correction trail, and do not merge unresolved screening and clinical records into a single beneficiary total.